جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نوال رمضان متولى عفيفى

المحافظة : القاهره **الحي / المدينة :** دار السلام **العنوان :** ==========

رقم البطاقة : 25612300100945 **رقم التليفون :** 01119084715 **تاريخ تسجيل الإستمارة :** 15-10-2026

العمر(تاريخ الميلاد) : 30-12-1956 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) التوقيع:(----------)

التشخيص الإكلينيكي : سرطان المعده

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان المعده

خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6131] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نوال رمضان متولي عفيفي
National ID: 25612300100945

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of gastric cancer for Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



نوال
رمضان متولى عفيفى
٤٦ ش عاشور سكر-جزيرة دارالسلام
دار السلام - القاهره

١٩٥٦/١٢/٣٠ ٢٥٦١٢٣٠٠١٠٠٩٤٥

JI1036825

٢٥٦١٢٣٠٠١٠٠٩٤٥ ٢٠٢٢/٠٤
ربة منزل

أنثى مسلمة متزوجة
حنفى حسن حنفى حسن
١ البطاقة سارية حتى ٢٠٢٩/٠٤/٠٥

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name : NAWAL RAMADAN

Date : 09-01-2020

POST ,CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS WITH CORONAL RECONSTRUCTION: -

Compared to last MR study dated 01-09-2019

Chest:

- Solitary 7 mm calcified nodule is seen at right lower lung lobe.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Evidence of thyroidectomy with residual part seen on right side.

Abdomen:

- Midline lower abdominal incision is noted with evidence of TAH & BSO showing clear operative bed with no evidence of locoregional masses or pathological lymph nodes.
- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- Few tiny dense stones are seen in the gall bladder.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Dorsolumbar spondylosis.

OPINION:

- ***Known case of cancer ovary, status post TAH & BSO and CTH; showing clear operative bed.***
- ***Fatty hepatomegaly.***
- ***Calcular gall bladder.***
- ***Right lower lobe calcified pulmonary nodule.***

DR. Ibrahim Elsedawy.M.Sc

Dr.m.gomaa,md

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name:Nawal Ramadan
Medical No. :

Date : 31/8/2019
Exam :pelviabdominal U/S

Department : Radiology

Referring Doctor :Adel Hassan

AbdominoPelvic Ultrasound

Liver: is enlarged with Bright homogenous echo pattern with normal internal structures. No discrete focal lesions or dilated biliary radicles. *P.V.* is patent and not dilated.

- *Gall bladder:* is of average size , normal wall thickness ,no stones or mud. *CBD* is not dilated.
- *Spleen:* is of average size with uniform texture and with no focal lesions.
- *Pancreas:* appears homogenous. *Para-aortic Area* is free.
- *Kidneys :* are average in size , site, shape and parenchymal pattern. No stones, cysts, or backpressure changes .
- *Urinary bladder*: is echo-free with no masses or stones and normal wall thickness .
- *Uterus*: is AVF and average in size, Myometrium is homogenous . no focal lesion .
- *Endometrial thickness = 9 mm .* → ? ?
- *Rt adenexal region shows a well defined cystic lesion with soft tissue component measuring 11.2 x 7.6 cm for further assessment .*
- No free fluid collection . → ? ?

Conclusion :

**Hepatomegaly with diffuse fatty infiltration for liver profile .*
**Rt Adenexal lesion as above .*

Much Obliged .
Dr.M.Hassan

Ultrasound
DIGITAL VIDEO ULTRASOUND

alfa
LABORATORIES

اسم المريض : الاستاذه / نوال رمضان متولي
رقم المريض : 17-43246
رقم الزيارة : 17190917480
عمر المريض : 63 Y

تاريخ الحضور : 01/09/2019
تاريخ التقرير : 01/09/2019

Immunology Unit

			Ref. Range
Tumor Marker: CA 125	**146.32**	U/mL	Less Than 35
Tumor Marker: CA 19-9	**3.75**	U/mL	Less Than 37

Comments :
CA 125 value must be interpreted in conjunction with information available from clinical evaluation and other diagnostic procedures.
CA 125 follow up is recommended.

Reviewed By:
Dr.Amal Abuelfotoh

Pt. Branch: المعادى1

Printed by: حسام عمر احمد

Printed on: 15:02 03/09/19

Call Center 16191

معامل الفا
A MEMBER OF ALFA MEDICAL GROUP
Lab Test
[[DOCTYPE.LAB_TEST]]

alfa
SCAN

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- Under filling of the urinary bladder appears distorted by the aforementioned lesion and the bulky uterus.
- No evidence of enlarged lymph nodes.
- Minimal pelvic free fluid.
- Preserved flow void character of iliac vessels, indicating patency.

OPINION:

- **Large predominantly cystic pelvic lesion with eccentric solid component, likely neoplastic of adnexal origin. For histopathological correlation.**
- **Bulky uterus showing increased endometrial thickness, for clinical correlation.**
- **Minimal pelvic free fluid.**

Dr. Amr Abdel Fattah, MD *Dr. Mahmoud Hanafy*

ID: 2189085

الفاسكان
MOHANDSSIN : El Safa Medical Tower - Hegaz Sq. Tel.: (202) 3336 3310
HELIOPOLIS : 73 Merghany Street. Tel.: (202) 2417 5556
MAADI : 150 Kornish El Nile - El Saudi Co. Bld. Tel.: (202) 2528 4888
OCTOBER : Sheikh Zaied- Zaied 2000, Arkan Mall. Tel.: (202) 3796 6256
NEW CAIRO : Cor... Mall - 90 Southern St. - 5th Settlement Tel.: (202) 2536 4050
Hotline: 16171

M.R.I
[[DOCTYPE:MRI]]

alfa
SCAN

Name: Nawal Ramadan Metwally

ID: 2189085
Date: 01/09/2019

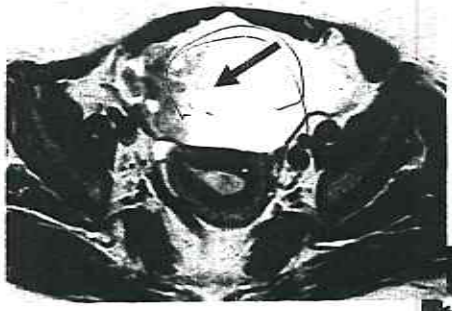
OPEN (1.0 TESLA)
MRI OF THE PELVIS

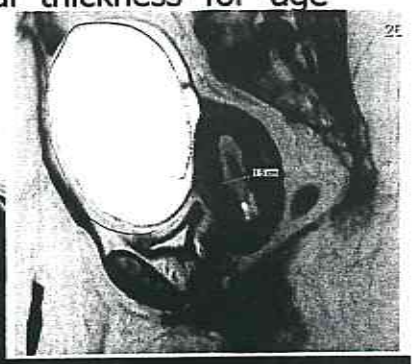
MRI TECHNIQUE:

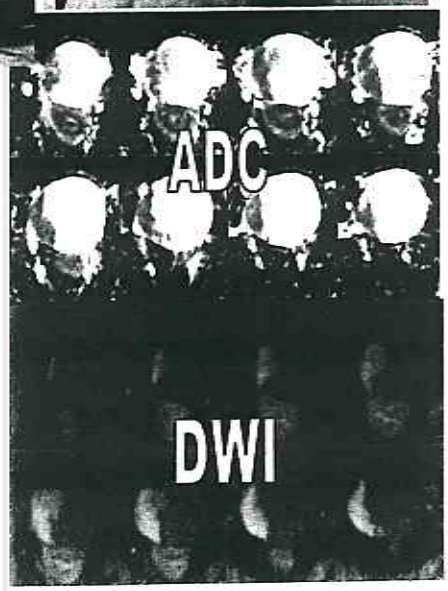
- **Sagittal 4 mm sections T2 WI** **(plate 1).**
- **Axial 4 mm sections T1 and T2 WI** **(plates 2, 3).**
- **Coronal 4 mm sections T1** **(plate 4).**

MRI FINDINGS:

- Bulky AVF uterus showing increased endometrial thickness for age measuring about 15 mm.
    - Few subendometrial and cervical cysts are noted.







- A large pelvic lesion predominant cystic nature is noted with eccentric polypoid solid component noted.
    - It measures about 11.5 X 8.5 X 12 cm along its maximum axial and craniocaudal dimensions respectively.
    - The predominant cystic component elicits bright T2 and low T1 signal while the solid component ***(arrow)*** displays intermediate to low T1 signal, intermediate T2 signal *with evident restriction of diffusion.*

الفا سكان
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NEW CAIRO : Concord Mall - 90 Southern St., 5th Settlement Tel.: [illegible] 4050
Hotline: 16171

M.R.I
[[DOCTYPE:MRI]]

Name: Mrs. Nawal Ramadan Metwally

Date: 7/9/2019 ***ID:2219027886*** ***Acc.:22190907107***

Referring Prof. Dr. Fathy Awad

Digital radiography examination of the chest (PA view), revealed:-

- Clear both lung fields and costophrenic angles.
- Normal cardiac size.
- Prominent aortic knob.

Truly yours

Dr. Ahmad Rashad, MD

Spelling checked by: Dr. sun

Typed: R.M. 07/09/19 8:06:45 PM

HOTLINE 19173
ACCREDITED MSCP-113
G-CERT
ISO 9001:2015
Client No: GIEG 0192-QC
Ultrasound
[[DOCTYPE:ULTRASOUND]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : نوال رمضان متولى **Analysis ID :** [redacted] 410805

Age : 63 Y **Gender :** Female

Referred By Prof. Dr. : فتحى عوض

Clinical Diagnosis : Right ovarian mass

Nature of Specimen : Total abdominal hysterectomy & left salpingeo-oophorectomy. Right salpingeo-oophorectomy and omentectomy

Receiving Date : 12/09/2019 **Patient ID :** 327151

PATHOLOGY REPORT

Gross :

Three specimens were received in the same container as:
- Total abdominal hysterectomy & left salpingeo-oophorectomy specimen,
Uterus measuring 6x5.5x3.5 cm. Endometrial thickness 0.4 cm. Myometrial thickness 3 cm and showed moderate trabeculations. Cervix 3.5 cm long, 2 cm in diameter with rubbery grayish white cut section, multiple mucin filled cysts. LT ovary measuring 2x1x0.7 cm, left tube measured 6.5 cm long.
- Right ovarian received separate, measured 11x9x4 cm, replaced by single unilocular parially cystic mass, the inner surface showd papillary fleshy pinkish growth, measured 10x7x4 cm, bulging on the outer surface.
- Omentum: Omental fatty tissue piece measured 40x14 cm, cut section was free of nodularities.

Microscopic :

Sections examined from the specimen received revealed endometrial wall showing thick endometrial lining with high glandular/stromal ratio showing endometrial glands with irregular and branching glands lined by malignant epithelial cells with marked anaplasia and bizzare nuclei.
No invasion of myometrium.
Right tubal wall showing mass formed of solid sheets, glands and papillae of markedly pleomorphic epithelial cells, Bizarre nuclei with high mitotic index. There is focal necrosis with extenisve infiltration of Right ovary.
The cervix revealed cervical tissue covered by mucous secreting columnar endocervical cells. The underlying substantia propria is fibrotic and showed inflammatory reaction and retention cysts. Free from tumour.
The left ovarian revealed cellular stroma and corpora albicans. Free left ovarian from tumour.
The left tube is unremarkable tube.
Sections from the omentum received revealed free congested omentum.

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ فاكس : ٢٥٧٧٣٢٥٩

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
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Nature of Specimen : Total abdominal hysterectomy & left salpingeo-oophorectomy, Right salpingeo-oophorectomy and omentectomy
Receiving Date : 12/09/2019
Patient ID : 327151

PATHOLOGY REPORT

Diagnosis :

Total abdominal hysterectomy & left salpingeo-oophorectomy, Right salpingeo-oophorectomy and omentectomy, ENDOMETRIAL HIGH GRADE SEROUS CARCINOMA, RIGHT TUBAL HIGH GRADE SEROUS CARCINOMA WITH RIGHT OVARIAN INVOLVEMENT, FREE CONGESTED OMENTUM, CERVIX FREE FROM TUMOUR, LEFT OVARY FREE FROM TUMOUR, UNREMARKABLE LEFT TUBE.
- ENDOMETRIAL STAGE: T1a.
- TUBAL STAGE: pT1.

16/09/2019

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ فاكس : ٢٥٧٧٣٢٥٦

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

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- ENDOMETRIAL STAGE: T1a.
- TUBAL STAGE: pT1.

16/09/2019

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور الرابع و الخامس و السابع
تليفون : ٢٥٧٩٠٩٣٥ - ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ فاكس : ٢٥٧٧٣٢٥٦

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

alfa SCAN

Name: Nawal Ramadan Metwally

ID: 2189085
Date: 01/09/2019

OPEN (1.0 TESLA) MRI OF THE PELVIS

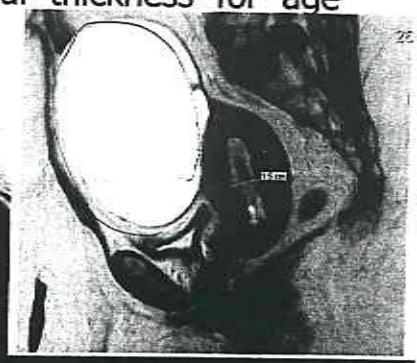
MRI TECHNIQUE:

- **Sagittal 4 mm sections T2 WI** **(plate 1).**
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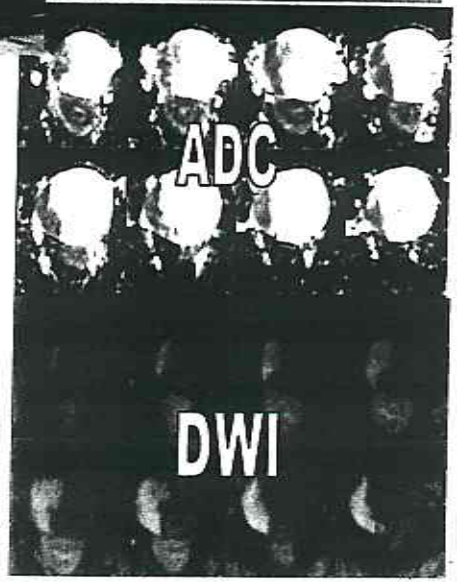
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